

KMH General Hospital

KM Hospitals International • NABH & JCI Accredited

Department of Cardiology & Cardiac Electrophysiology

Outpatient & Inpatient Services, OPD Wing 3

Dr. Arjun Venkataraman, MD, DM (Cardiology)

MBBS — Stanley Medical College | DM (Cardiology) —

AIIMS New Delhi

Reg. No.: MCI-CD-2007-02198

Tel: +91-44-6699-0000 • cliniq@kmhospital.org

PATIENT DISCHARGE SUMMARY & PRESCRIPTION REPORT

PATIENT DEMOGRAPHICS

PATIENT NAME Rajan Iyer	PATIENT ID P-00391
AGE / SEX 69 Yrs / Male	DATE OF BIRTH 14 September 1955
BLOOD GROUP A Positive	WARD / UNIT Cardiology Ward — Bed 14B
ADMIT DATE 22 June 2025	DISCHARGE DATE 29 June 2025
ATTENDING PHYSICIAN Dr. Arjun Venkataraman, MD, DM (Cardiology)	MED. REG. NO. MCI-CD-2007-02198

ALLERGY / ADR ALERT:

- Penicillin — anaphylaxis (documented 2011, ER admission)
- Aspirin — GI intolerance, haematemesis (2019)

CURRENT VITALS & BIOMETRICS (ON DISCHARGE)

PARAMETER	RESULT	UNITS	REFERENCE	STATUS
Blood Pressure (Sitting, Right Arm)	158/96	mmHg	< 130/80	HIGH
Heart Rate	92	bpm	60 - 100	Normal
Fasting Plasma Glucose	212	mg/dL	70 - 99	HIGH
HbA1c (Glycated Haemoglobin)	9.1	%	< 5.7	HIGH
Temperature	98.9	F	97.8 - 99.1	Normal
SpO2 (Pulse Oximetry)	94	%	>= 95	LOW
LDL Cholesterol	162	mg/dL	< 70 (high risk)	HIGH
Serum Creatinine	1.4	mg/dL	0.7 - 1.3	HIGH
Serum Potassium	3.3	mEq/L	3.5 - 5.0	LOW

DIAGNOSES / ACTIVE PROBLEM LIST

ICD-10	DIAGNOSIS	STATUS	SINCE
I48.11	Paroxysmal Atrial Fibrillation	Active	Mar 2023
E11.65	Type 2 Diabetes Mellitus with hyperglycaemia	Active	Jan 2015
I10	Essential Hypertension, uncontrolled	Active	Apr 2012

E78.01	Familial Hypercholesterolaemia (Pure)	Active	Jan 2015
--------	---------------------------------------	--------	----------

CONSULTATION BRIEF / CLINICAL ASSESSMENT

Patient Rajan Iyer, a 69-year-old male with known paroxysmal atrial fibrillation, Type 2 diabetes, hypertension, and familial hypercholesterolaemia, was admitted on 22 June 2025 following a 6-hour episode of palpitations, mild breathlessness on exertion, and dizziness. ECG on admission confirmed AF with rapid ventricular response (HR 118 bpm). Rate control achieved with IV Diltiazem infusion; patient reverted to sinus rhythm on Day 2. Echocardiogram showed mild left ventricular hypertrophy with preserved EF (58%). Blood pressure poorly controlled throughout admission (persistent readings >150/90 despite current regimen). Fasting glucose 212 mg/dL and HbA1c 9.1% indicate significantly uncontrolled diabetes; insulin initiation deferred pending Endocrinology outpatient review. Aspirin allergy documented — anticoagulation with Apixaban (NOAC) initiated for stroke prevention (CHA2DS2-VASc score: 5). Patient is non-compliant with prior antihypertensive therapy by self-report. Medication regime rationalised and patient extensively counselled on adherence, low-sodium diet, and cardiac warning signs. Discharged stable in sinus rhythm.

RX ACTIVE PRESCRIPTION REGIMEN

#	MEDICATION (GENERIC NAME)	DOSE	FREQUENCY	DURATION	PILL DESCRIPTION
1	Apixaban <i>Apixaban 5mg (NOAC — Factor Xa Inhibitor)</i> Take with or without food. Do NOT miss doses — increases stroke risk in AF. WARNING: Do NOT combine with NSAIDs, Aspirin, or other anticoagulants — major bleeding risk. Avoid grapefruit.	5 mg	Twice Daily	Ongoing	<i>Small, round, pale yellow tablet marked '894' on one face. Film-coated.</i>
2	Metoprolol Succinate XR <i>Metoprolol Succinate 100mg (Beta-1 Blocker, Extended Release)</i> Take at the same time each morning. Do NOT crush, chew or split the extended-release tablet. Do NOT stop abruptly.	100 mg	Once Daily (morning)	90 Days	<i>White oblong tablet, scored on one side, marked 'A/mS' with score line.</i>
3	Amlodipine Besylate <i>Amlodipine 5mg (Calcium Channel Blocker)</i> Take at fixed morning time. May cause peripheral oedema — monitor lower limbs. Avoid grapefruit juice.	5 mg	Once Daily (morning)	90 Days	<i>Small, pale yellow-white oval tablet. Approximately 7mm. Marked 'AML5'.</i>
4	Atorvastatin Calcium <i>Atorvastatin 80mg (High-Intensity Statin)</i> Take at bedtime — cholesterol synthesis peaks nocturnally. Report any unexplained muscle pain or dark urine immediately. WARNING: Report myopathy symptoms immediately. Avoid Clarithromycin/Erythromycin — rhabdomyolysis risk.	80 mg	Once Daily (night — bedtime)	90 Days	<i>Oblong white tablet marked '80' on one face. Film-coated. Approximately 14mm.</i>

- Take all medications exactly as directed. Do NOT self-adjust doses.
- Carry this discharge summary and medication list to all follow-up appointments.
- Return to the Emergency Department if: chest pain, severe breathlessness, sudden weakness, persistent fever >101F, or unusual bleeding.
- Avoid NSAIDs, ibuprofen, and naproxen unless explicitly approved by your treating physician.
- Attend scheduled follow-up on 28 July 2025 — bring all current medications.

Reg. No.: MCI-CD-2007-02198

Next Review: 28 July 2025

This document is electronically generated via ClinIQ Physician AI System — KMH General Hospital. CONFIDENTIAL — subject to DPDPA 2023 and patient privacy regulations. Valid for medico-legal and pharmacy use for 90 days from discharge date.